



**LITTLE ROCK, AR
BUILD-TO-SUIT SINGLE TENANT HEALTHCARE FACILITY
PREFERRED EQUITY OPPORTUNITY**

\$37.9M
TOTAL
INVESTMENT

3-4 YEAR
EST. HOLD
PERIOD

\$30.3M
CONSTRUCTION
LOAN

\$7.6M
PREF EQUITY

15%
PREFERRED
RETURN

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If either party does not wish to pursue negotiations leading to the placement of the Investment, you agree to return this Confidential Submission to Cross.

Table Of Contents

- Executive Summary
- Investment Highlights
- Key Considerations
- Tenant & Manager Overview
- Market & Location Overview
- Aerials & Site Plans
- Opportunity Financials

Executive Summary



Executive Summary

Cross Development and Kennor Holdings (“Cross” or “Sponsor”) have been engaged by Nobis Rehabilitation Partners for the development of a 48 bed, 61,819 sq. ft. Inpatient Rehabilitation Facility (IRF) (the "Subject Property") in Little Rock, AR. The Subject Property is **100% leased** to the operator of the hospital, an entity owned and managed by Nobis Rehabilitation Partners, LLC. Cross is in the process of securing an **80% LTC construction loan** in the amount of **\$30,339,000**. Cross will raise an additional **\$7,585,000** as equity to complete the capital stack. The equity raise will include **\$1,896,000** of Sponsor contribution. The Preferred Equity will receive a guaranteed **15% IRR** for their investment. The 15% return will be guaranteed by Cross Development’s ownership interest in Nobis Rehabilitation Partners. Cross is developing the asset to a build-to cap rate of **10.0%** and intends to sell the asset at an anticipated exit cap rate of 7.00%, which results in a 300 basis point development margin. This project is the seventh IRF in the series of build-to-suit properties that Cross has secured with Nobis as its preferred development partner. The project is expected to be completed in 18 months from groundbreaking, with an anticipated opening in **Q3 2027**.

The tenant, Little Rock Rehabilitation Hospital, LLC (“Tenant”), will provide highly experienced, quality care and treatment for patients with a wide variety of rehabilitation needs. Treatment and rehabilitation needs vary from brain related trauma, amputation, and mental health services. They have a proven track record of success helping individuals achieve sustained recovery. The Subject Property includes a new 20-year fully guaranteed lease with 1.5% annual escalations that continue through the two, five-year options. The expense structure of the lease is Absolute NNN with no landlord obligations. The Year 1 net operating income of the asset will be \$3,792,386.

The hospital operator, Nobis Rehabilitation Partners, LLC, is committed to enabling hospitals to perform at the highest level possible. They are dedicated to finding the operational efficiencies and optimization strategies required to care for patients with life-altering injuries. Nobis looks to continue their national expansion. The existing Nobis operating portfolio consists of 16 hospitals nationwide.

Since inception in 2003, Cross has developed over 600 properties totaling 6 million square feet with an estimated value of over \$2,000,000,000. Cross has 30 full-time employees spread out between offices in Tampa, and Dallas, their headquarters. They have developed property in 32 states with approximately 20 projects currently under construction or development. In addition to single-tenant construction, Cross is also experienced in developing multi-family, storage, and car washes.

Development Attributes	
Address	6301 District Avenue, Little Rock, AR 72205
MSA	Little Rock, AR
Construction Commencement	Q4 2025
Licensed Beds	48
Size (acres)	2.9 acres
Building Size	61,819
Guarantor	Nobis
Use	Inpatient Rehabilitation Hospital
Term of Loan	5 Years
Construction Loan	\$30.3M
Equity Raise	\$5.7M
Sponsor Contribution	\$1.9M
Total Projet Cost	\$37.9M
Est. Hold Period	3 - 4 Years



Project Breakdown

Cross will develop the IRF project, which will be managed by Nobis Rehabilitation Partners, LLC. Nobis’s initial lease will account for base rent at 10.00% of the total development costs and escalate annually at 1.50%. Cross is expecting to commence construction in **Q4 2025** with operations beginning in **Q3 2027**.



Key Market Studies



Tenant: **Little Rock Rehabilitation Hospital, LLC**



Lease Term: **20 Years**



Abatement: **3 Months**



Year 1 Rent: **\$3,792,386 | 10.0% YTC**



Absolute NNN w/ **1.50% Annual Escalations**



Full Parent Company Guaranty

Development Summary



Developer: **Cross Development**



Total Beds: **48**



Construction Budget: **±\$37.9 Million**



Construction Time: **18 Months**



Project Size: **61,819 SF**

Investment Highlights



NEW BUILD, INSTITUTIONAL QUALITY ASSETS

- Built-to-suit, state-of-the-art facility providing long-term utility and functionality to the Tenant
- Developed by a nationally renowned real estate developer with long track record of quality design and construction
- Planned for optimized physical design and operational functionality with direct input from Manager



HIGHLY-REGARDED INPATIENT REHABILITATION MANAGER / OPERATOR / GUARANTOR

- Renowned inpatient rehabilitation operator / manager led by former hospital system C-suite executives
- Experienced Operator, having developed, opened, and/or managed over 35 IRFs and acute care hospitals over the past 30 years
- Nobis's current portfolio with the Sponsor group has showcased rapid success, operating profitably at near capacity



INVESTOR-FRIENDLY LEASES WITH ZERO MAINTENANCE & CORPORATE LEASE GUARANTY

- Percentage of budget-based yield constant provides protection against final costs
- 20-year lease term ensures long-term contractual cash flow and protection against inflation risk
- New developments with an absolute triple-net lease, requiring zero upkeep or maintenance



SIGNIFICANT LONG-TERM VIABILITY AND GROWTH IN IRF SPACE

- Standalone IRFs play vital and expanding role in health care by providing a lower cost of care setting and better outcomes
- Shift to PDPM in skilled nursing facilities reduced revenue for therapy services thus pushing rehabilitation services to IRF settings
- Wide range of services provides diversity in patient-type and ability to provide treatment for medium-to-high acuity conditions



STRATEGIC LOCATIONS WITHIN CLOSE PROXIMITY TO HOSPITALS FOR AMPLE REFERRALS

- Dominant market position with minimal freestanding IRF competition
- Property is located within two miles of four major hospitals in Little Rock (approx. 2,300+ beds). This accounts for over 53% of beds in the Little Rock MSA
- Positioned to be the first freestanding IRF in the Midtown Medical Corridor

Key Considerations

TOP-TIER SPONSOR WITH ESTABLISHED TRACK RECORD

Cross Development and Kennor Holding are highly experienced developers with 22 years and 17 year of experience, respectively. Leadership at both groups have collectively developed over hundreds of sites exceeding \$2.5B in value. Both groups have extensive experience developing specialty healthcare facilities, and this site is their seventh Nobis Hospital together.

PROVEN INPATIENT REHABILITATION OPERATOR

Founded in 2018 and led by Chester Crouch, Nobis has grown to operate 16 facilities across 10 states. Prior to launching Nobis, Chester led Reliant Rehabilitation Hospitals, where he grew the company to 15 hospitals and sold to HealthSouth [now Encompass Health Corporation - NYSE: EHC] for \$730MM. Mr. Crouch is regarded as the best-in-class IRF manager in the U.S.

Nobis is a leader in freestanding facilities, helping patients regain independence after an acute event or illness, musculoskeletal injury, surgery or hospitalization. Care is provided by an interdisciplinary team including physical, speech and occupational therapists, physiatrists, physician specialists, rehabilitation nurses, case managers and dietitians, among others. Patients receive at >15 hours of therapy five days per week while under care of registered nurses with daily physician visits.

MEDICAL ISSUES COVERED AT NOBIS FACILITIES

STROKE & BRAIN INJURY	NEUROLOGICAL DISORDERS	SPINAL CORD INJURY
MEDICALLY DEBILITATING CONDITIONS	MAJOR MULTIPLE TRAUMA	ORTHOPEDIC INJURY
CARDIO-PULMONARY	AMPUTATION	ONCOLOGY



Key Considerations

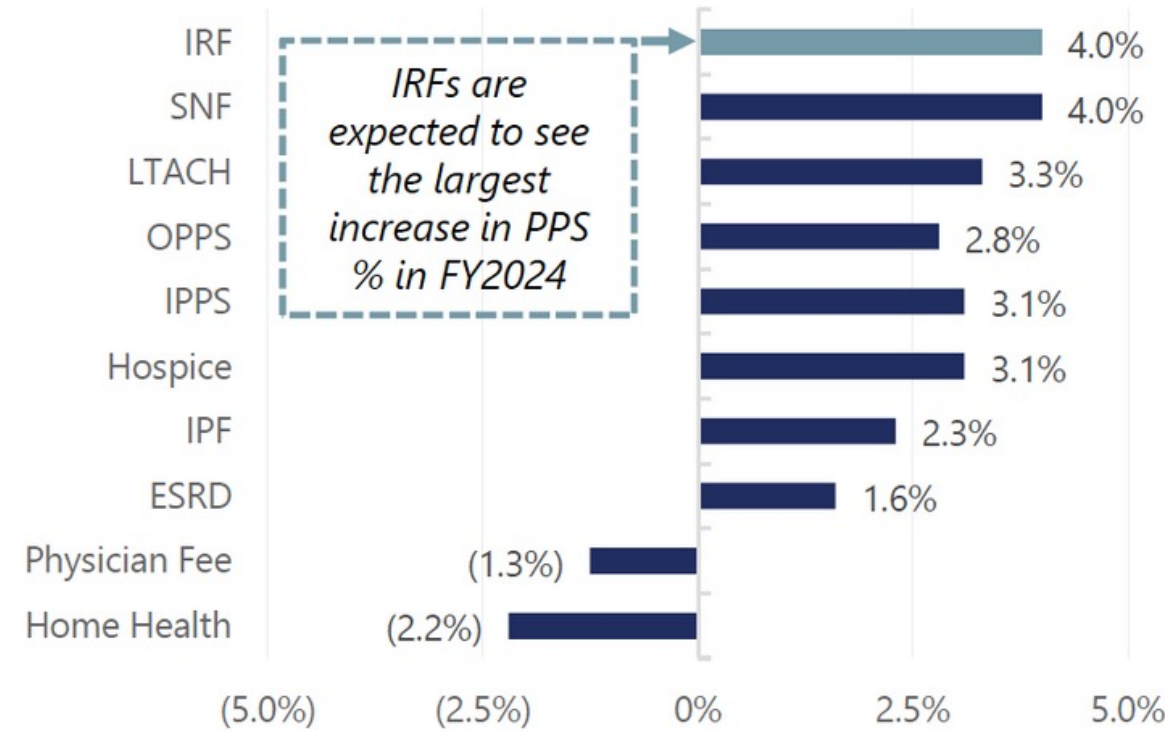
ACCELERATING IRF DEMAND

The shift to the Patient Driven Payment Model (“PDPM”) for skilled nursing facilities that went into effect October 1, 2019, is fueling investment demand in the IRF space. PDPM pays lower rates for patients needing therapy and higher rates for patients needing complex nursing care. In response, skilled nursing facilities laid off therapists across the country and diminished the therapy provided as they are incentivized to admit patients who use ventilators and/or have complex medical needs. Prior to PDPM, skilled nursing facilities provided therapy intensity similar to an IRF, in order to receive higher reimbursement, but currently there is no longer a financial incentive to do so, which has proved to be an economic tailwind to the inpatient rehabilitation space.

FAVORABLE REIMBURSEMENT TAILWINDS

Centers for Medicare & Medicaid Services (“CMS”) has initiated updates to the Prospective Payment System (“PPS”) and rates, recalibrating the IRF market basket to use 2021 as the baseline after evaluating FY 2022 IRF claims along with 2021 IRF cost report data. The market is poised for a 4.0% increase in national IRF reimbursements, comprising a 3.4% IRF market basket update for FY 2024 and an approximated 0.6% uplift from IRF outlier payment adjustments. Effective October 1, 2023, and notably outpacing all other healthcare verticals by over 21%, barring SNFs, the revised payment system is forecasted to funnel an aggregate of \$336 million in reimbursements to IRF Medicare providers.

CMS 2024 Proposed Payment Update



Key Considerations

LOCATION WITHIN MEDICAL CORRIDOR NEAR FOUR MAJOR HEALTH SYSTEMS

The Property, centrally located in the Midtown Medical Corridor of Little Rock, Arkansas, offers unparalleled access to 4 major hospitals within a 2 mile radius. The site also has close proximity to an additional 500,000+ SF of medical office space, the University of Arkansas for Medical Sciences (UAMS) and Arkansas Children's Hospital. This density of medical uses serves as an abundant referral source for post-acute care. Despite the medical activity in this trade area, there are currently no freestanding IRF's in the Midtown Medical Corridor, presenting Nobis with a strong competitive advantage.

The vicinity includes healthcare, retail and residential areas catering to over 150,000 people within a 5-mile radius. Access is via I-630, which sees an average daily traffic of 112,000 vehicles and N University Ave, which sees an additional 33,000 vehicles per day. The trade area is also home to over 1 million SF of retail, including Park Plaza Mall, as well as a variety of regional and necessity-based shopping centers that drive consistent traffic to the market.

Within a 2.0-mile radius of the Property are four major hospitals: Baptist Health Medical Center (834 beds), UAMS Medical Center (514 beds), CHI St. Vincent Infirmary (615 beds) and John L McClellan Memorial Veterans Hospital (280 beds). They provide acute care services and are easily accessible via Interstate 630. The four hospitals account for ~53% of the total beds in the Little Rock primary market area. Strategically developed in a retail-centric location within a medical corridor will enhance the development's potential for long term-success.



Tenant & Manager Overview



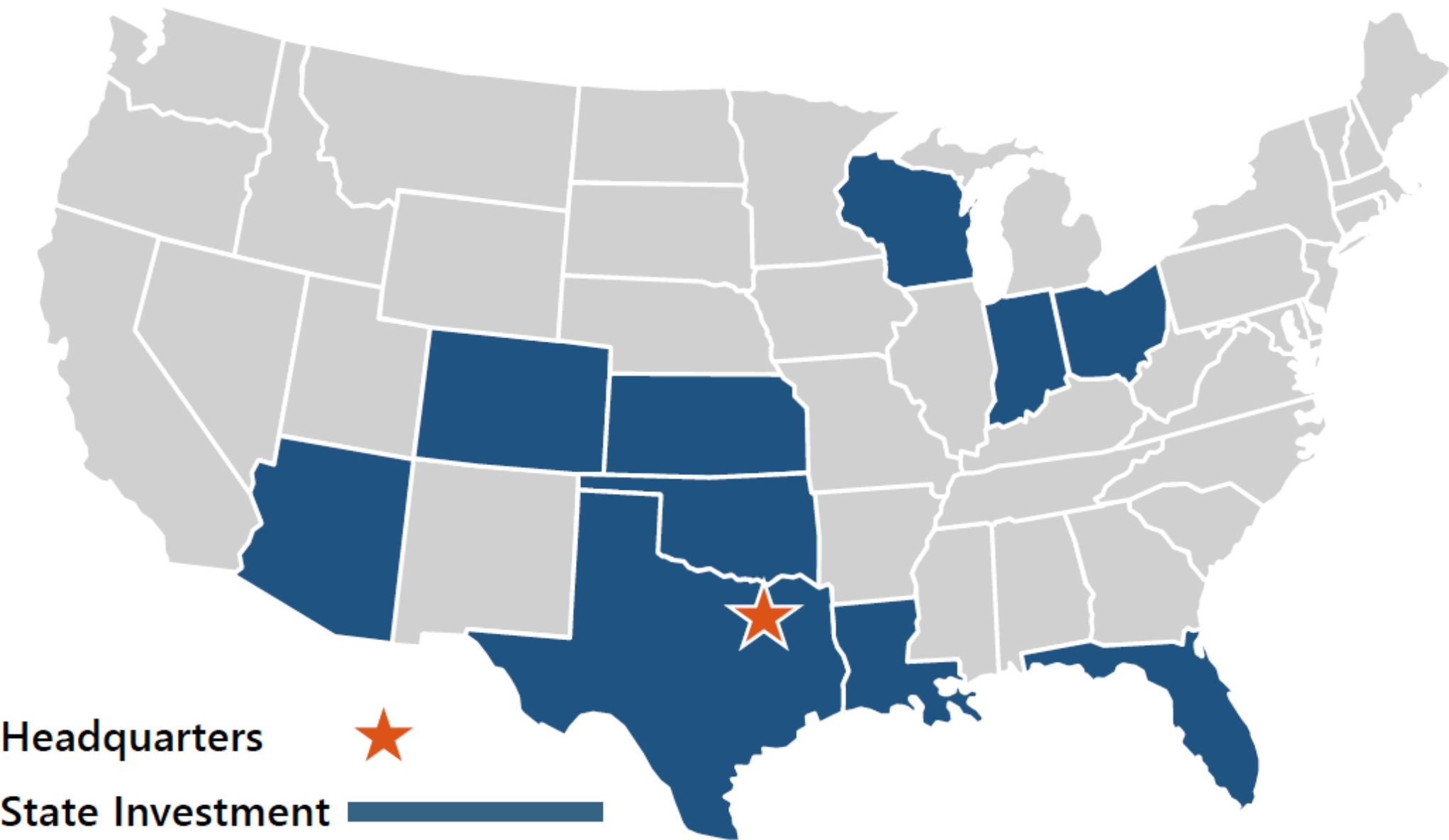
IRF Manager & Tenant | Nobis

Nobis is a rehabilitation management firm that owns and manages hospitals, aiming to deliver goal-directed care through individualized, comprehensive treatment plans and intensive therapeutic interventions.

Overview

- Established in 2018, Nobis Rehabilitation Partners, LLC is managed by Chester Crouch (Founder & CEO), Jerry Huggler (CFO), and Christopher Bergh (COO). Dedicated to managing hospitals and hospital-based units.
 - Leadership team has developed, opened, and/or managed over 35 IRFs and acute care hospitals over the past 30 years.
 - Chester Crouch formerly led Reliant Rehabilitation which sold to HealthSouth [now EHC] for \$730MM.
- In growth mode to manage newly-developed rehab hospitals, manage rehab units and partner with health systems around their rehabilitation service lines.
- Sixteen IRFs under Nobis management, including three hospitals owned by Nobis.
- Three Nobis IRFs under construction with three additional Nobis IRFs (including Little Rock) breaking ground in 2025.

Operating Map - Nobis Rehabilitation Partners Locations



IRF Manager | Nobis Rehabilitation Partners, LLC



Chester Crouch

Founder & CEO

- Founded Nobis Rehabilitation Partners in 2018.
- 25+ years of healthcare experience.
- Previously Co-Founded Reliant Hospital Partners.
- Prior experience as CEO of Curahealth Hospitals.



Jerry Huggler

Chief Financial Officer

- 20+ years of experience in acute and post-acute organizations.
- Over \$200M of healthcare acquisition experience.
- Prior experience as CFO of North Texas Hospital and Reliant Hospital Partners.



Christopher Bergh

Chief Operating Officer

- 25+ years of healthcare experience.
- Responsible for executing growth and performance strategies.
- Previously served as EVP of Operations at Ernest Health.



Gina Thomas

Chief Development Officer & Public Relations Officer

- 40+ years of healthcare experience.
- Serves on the Clinical Advisory Board at Florence Health.
- Served in clinical roles at 11 different healthcare organizations.

Sponsor Overview | Cross Development

Cross Development has successfully completed over 400 customer locations in the past five years and is actively pursuing new opportunities. Cross's most recent strategy involves investment in the IRF space as the Company has seen great success across its three open facilities in Oklahoma City and Tulsa, OK as well as Blue Ash, OH

Overview

- Headquartered in Carrollton, TX, Cross Development is an established development firm specializing in single-tenant commercial properties.
- Offer vertically integrated development services across all real estate asset classes.
 - Example developments include hospitals, ER facilities, car washes, residential, grocery stores, automotive repair canters and more.
- Developed and acquired over 600 properties with over \$2.0 billion in investments and development across 30+ states.
- Recent investment strategy to deploy inpatient rehabilitation facilities in areas of rising demand in partnership with Nobis Rehabilitation Partners, LLC.
 - Three developed with three under construction and breaking ground on three more by the end of 2025

 Founded in **2003**

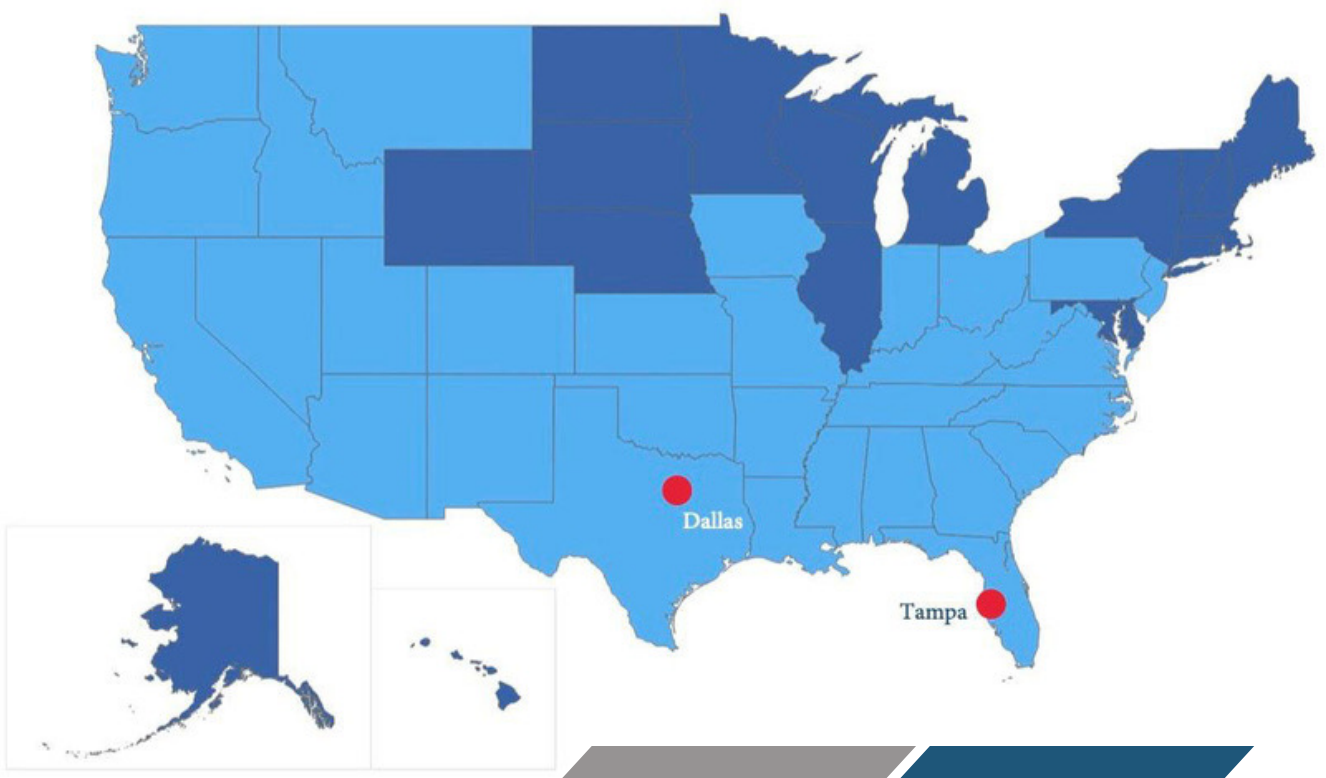
 Headquarters: **Carrollton, TX**

 Properties Acquired or Developed: **600+**

 Employees: **30+**

 Total Value of Developments: **\$2.0 Billion+**

Real Estate Developments & Investments



Partnerships & Experience

Healthcare	Retail
	 
	 
	 
	 
	 
	 
	 

Sponsor | Cross Development & Kennor Holdings

Deno Maggi | Partner

Deno joined Cross Development after years of developing, managing, and owning many different real estate asset classes for his own companies. He is a graduate of the University of Oklahoma with a degree in Economics. Deno currently oversees the overall operations and development for several of Cross' valued clients and special projects.



Steve Rumsey | Partner

Steve Rumsey co-founded Cross Development in 2003. He brought to the development company an in-depth knowledge of architectural design, construction and cost analysis for retail project development. His past experience enables him to specialize in “Build to Suit” retail development with a focus on the development process from site acquisition to tenant satisfaction and acceptance of a building built to their exact specification.



Casey Shires | Partner

Casey Shires co-founded Cross Development in 2003 to execute a complete real estate process and a transparent model that delivers projects on time for specific retailers. From day one, it was Casey's vision to deliver a company committed to delivering real estate development services and solutions unsurpassed in our industry.



Shane Shoulders | Partner at Kennor Holdings

Shane Shoulders is the founding partner of Kennor Holdings, a family of commercial real estate development companies with a portfolio of regional and national projects based in Dallas, TX. Shane has more than 15 years of commercial real estate development experience and has structured syndication in excess of \$300M. His company, Kennor Holdings, specializes in offering clients a full spectrum of commercial real estate development services from project launch to completion. He has experience in a variety of asset classes including industrial, multifamily, retail, and medical.



Joe Dell | CEO

Joe joined Cross Development in 2014 as a Development Manager. Prior to joining Cross, he spent 10 years in both residential and retail development gaining a vast knowledge base of construction, cost analysis and site planning. He has played a vital role in Cross' growth and held several positions before being promoted to CEO in June of 2023. In his role as CEO, Joe oversees financing, development, construction and company operations.



Stephen Swann | Vice President of Capital Markets

Stephen Swann joined Cross Development in January 2023 as Vice President of Capital Markets. During Stephen's 25 years in commercial real estate, he has owned, managed, and brokered a diverse portfolio of retail, industrial, multi-family as well as office properties. He is a graduate of the University of Oklahoma with a BBA in Finance. Additionally, Stephen is an alum of the Harvard Graduate School of Design Advanced Management Development Program.



IRF Market Overview & Location Overview



The U.S. healthcare industry is the third-largest in the nation, impacting the life of nearly every American. The United States spends more on healthcare per citizen than any other nation in the world.

- Healthcare represented over 18% of the U.S. economy in 2021, over \$4.3 trillion in revenue
- 14% of U.S. adults are employed in the healthcare industry
- An aging population of Baby Boomers is creating increased demand for healthcare services. The average American over 65 years old will require an estimated \$315,000 for medical expenses during retirement.
- In 2011, the Baby Boomer generation became eligible for medicare.
- In 2019, the Baby Boomer population was 54 million. In the next ten years, it is estimated to surpass 77 million
- The U.S. Skilled Nursing and Rehabilitation market was valued at over \$227B in 2022. Market growth is expected to maintain a CAGR of nearly 4% from 2023 to 2030.

The healthcare industry is well positioned for continued exponential growth due to technological advances, new and dynamic treatments, and existing facility obsolescence. Modernized hospital operations will become increasingly valuable as post-pandemic healthcare continues to evolve, including the development of more sophisticated Telehealth and virtual options for prospective patients.

Healthcare Legislation

The Centers for Medicare and Medicaid Services ("CMS") has issued an update regarding payment policies for Inpatient Rehabilitation Hospitals. Next year, CMS is proposing an update to Inpatient Rehabilitation Facility Prospective Payment System payment rates by 3%. The proposed rule adjustment also includes an outlier threshold to keep outlier payments at 3% of total payment balance, resulting in increased outlier payments by 0.7%. CMS is estimating that Inpatient Rehabilitation Facility payments will continue to increase over the next several years, reaching \$335 million by the end of 2023.

Under the new legislation, Inpatient Rehabilitation Hospitals will be allowed to open and begin being paid under IRF PPS at any point during the reporting period. CMS hopes that the updated guidance will alleviate reporting complexity and increase access and quality of care.

IRF Market Overview

Inpatient Rehabilitation Facilities are freestanding hospitals that receive discharged patients from acute care hospitals in need of intensive (nonchronic) rehab care after a traumatic injury or covered condition.

Understanding Inpatient Rehabilitation

- Central focus of inpatient rehabilitation facilities is to provide treatment for medium-to-high acuity conditions through a combination of intensive rehabilitation and high-level clinical supervision.
 - Conditions treated by IRF include traumatic brain injuries, spinal cord injuries, cancer, organ transplants, major burns, complex trauma, strokes, and other neurological and orthopedic conditions.
- IRFs typically are reimbursed for their costs via Medicare or private insurance.
 - Annually, +/-75% of IRF revenues come from Medicare.
 - A significant portion of IRF patients are seniors (65+).
- 60% Rule: 60% of IRF's patients must require treatment for at least one of 13 conditions(2).

IRF Market in the US

- There are approximately 350 IRFs within the U.S. as of Apr-2023.
 - This excludes inpatient rehabilitation units located in acute care hospitals.
- Freestanding units report a greater all-payer margin at ~10.5%, compared with hospital-based facilities at ~7.0%(1).
- Of the ~350 operating IRFs, over 80% are operated by non-forprofit operators.
- Since 2006, the percentage of rehabilitation beds located inside hospitals [units] has decreased by 75%. Two biggest rationale:
 - (1) Better patient outcomes
 - (2) Reduction in costs to operate
 - (3) Partnerships with for-profit IRF providers

Key Market Studies



Market Size in 2023: **±\$10 Billion**



Total Facilities in Sector: **~350**



IRF Medicare Spending: **\$8.0 Billion**



Total Beds in Sector: **+25,000**

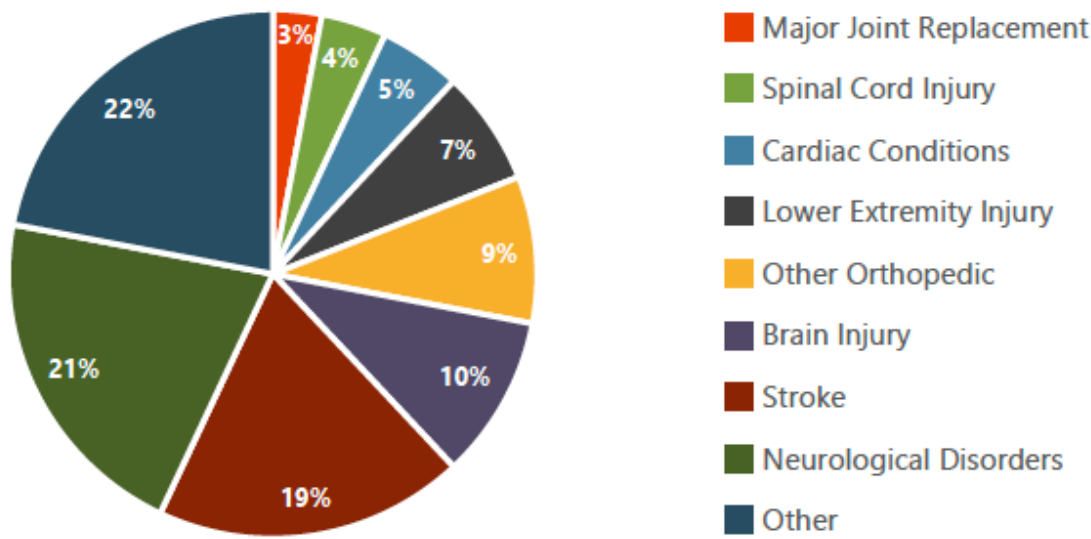
Key Market Studies

- The U.S. Inpatient Rehabilitation Facility (IRF) industry provides services to patients who need intensive rehabilitation care typically following an injury, illness, or surgical procedure.
- IRFs are licensed as rehabilitation hospitals -> must comply with hospital building codes and are federally (CMS) certified and state certified.
- Geographically, IRFs are concentrated in areas with large Medicare populations given that Medicare is generally the largest payor for IRF services.
- The industry is mostly nonprofit and urban, and comprised of ~31% freestanding facilities and ~69% hospital-based units.

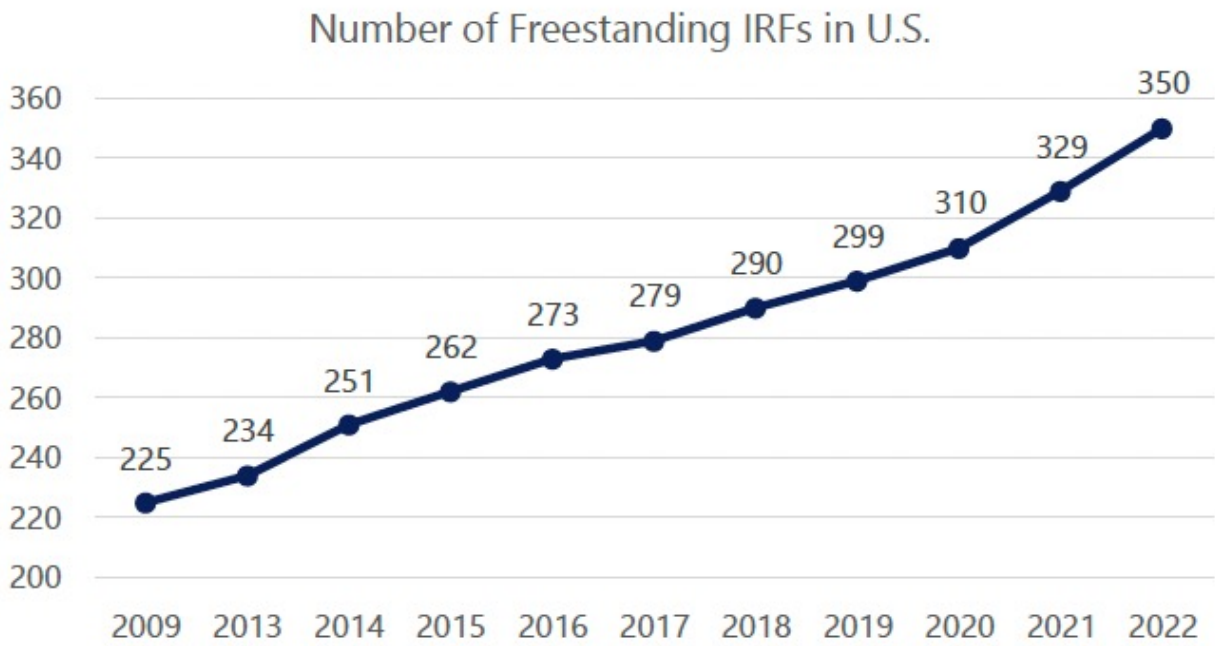
IRF Market Overview

Inpatient Rehabilitation Facilities are freestanding hospitals that receive discharged patients from acute care hospitals in need of intensive (nonchronic) rehab care after a traumatic injury or covered condition. IRFs on average have a significantly shorter length of stay, sometimes ranging from 10-20 days, when compared to Skilled Nursing Facilities (“SNF”).

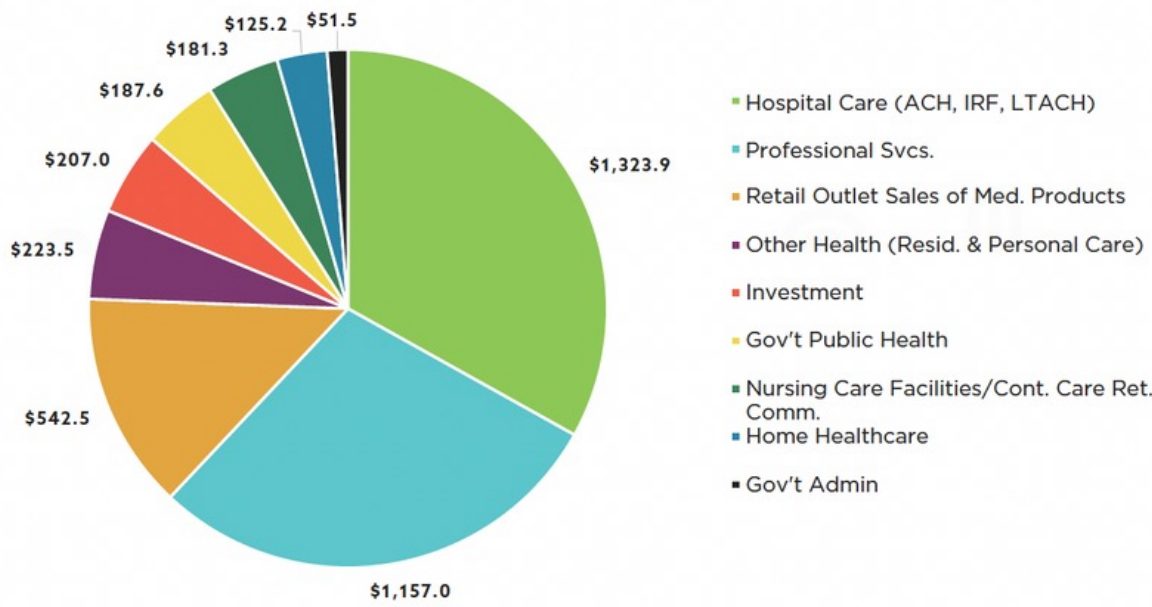
IRF Patient Case Mix (2021)



IRF Steadily Growing in Post-Acute Care



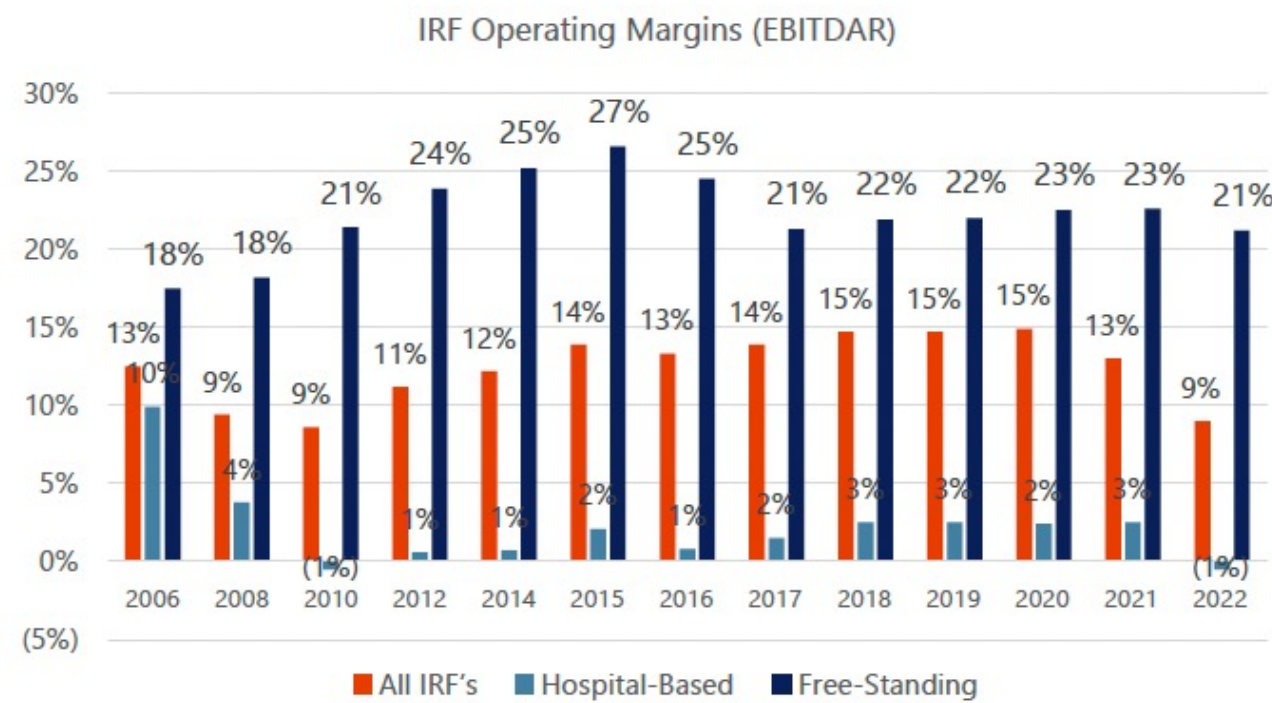
Allocation of 2021 Nat’l Healthcare Spending (in Billions)



Note: *Excludes Net Cost of Private Health Insurance of \$255.7 billion

Centers for Medicare & Medicaid Services, National Health Expenditure Data (Historical), Table 2-2021.

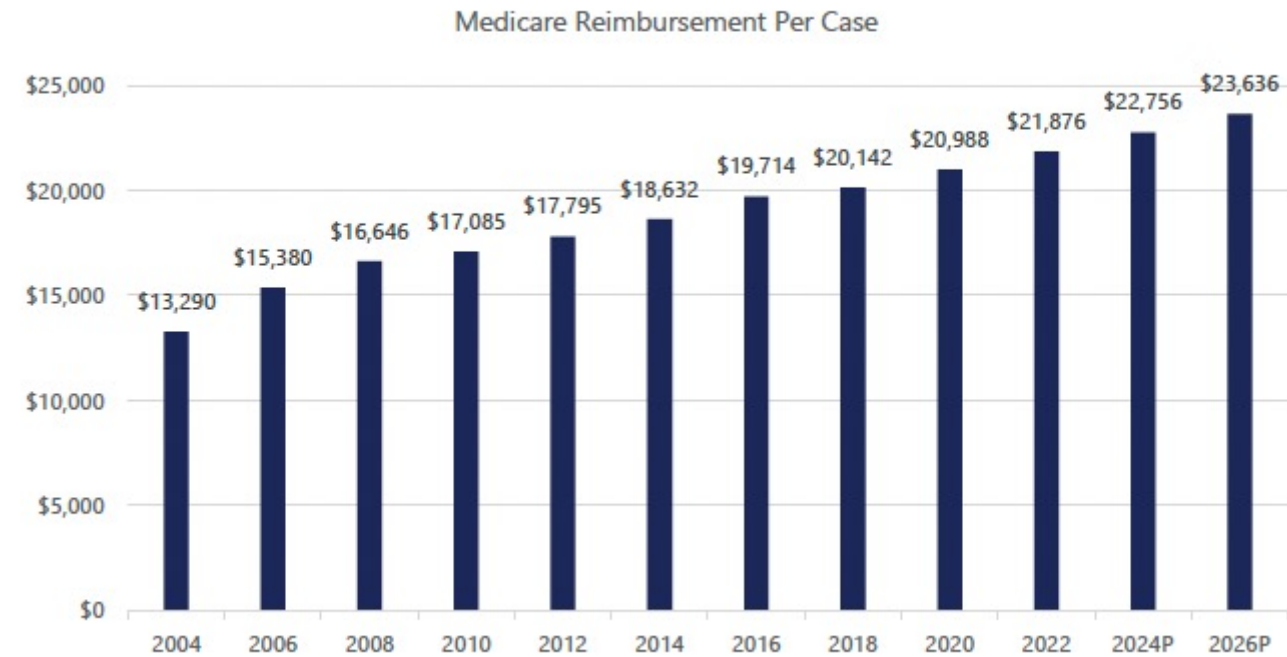
IRF Margins - Hospital-Based vs. Freestanding Facilities (EBITDAR)



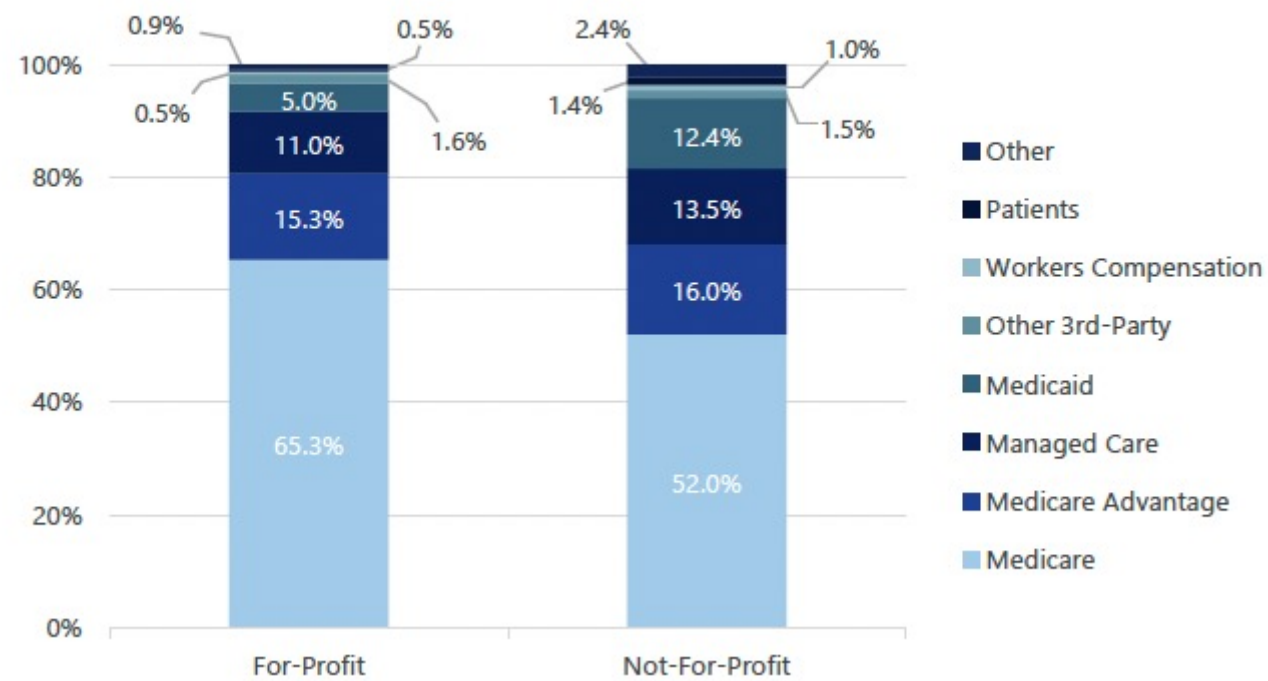
IRF Market Overview

Inpatient Rehabilitation Facilities have the physical construct, clinical staffing and operating expertise to “pivot from the center” to address the full spectrum of inpatient post-acute needs in a site neutral environment. No other healthcare setting offers IRFs’ specialized programming for hospital level patients requiring intensive rehabilitation.

Payments Per Case Have Increased Every Year Since 2004



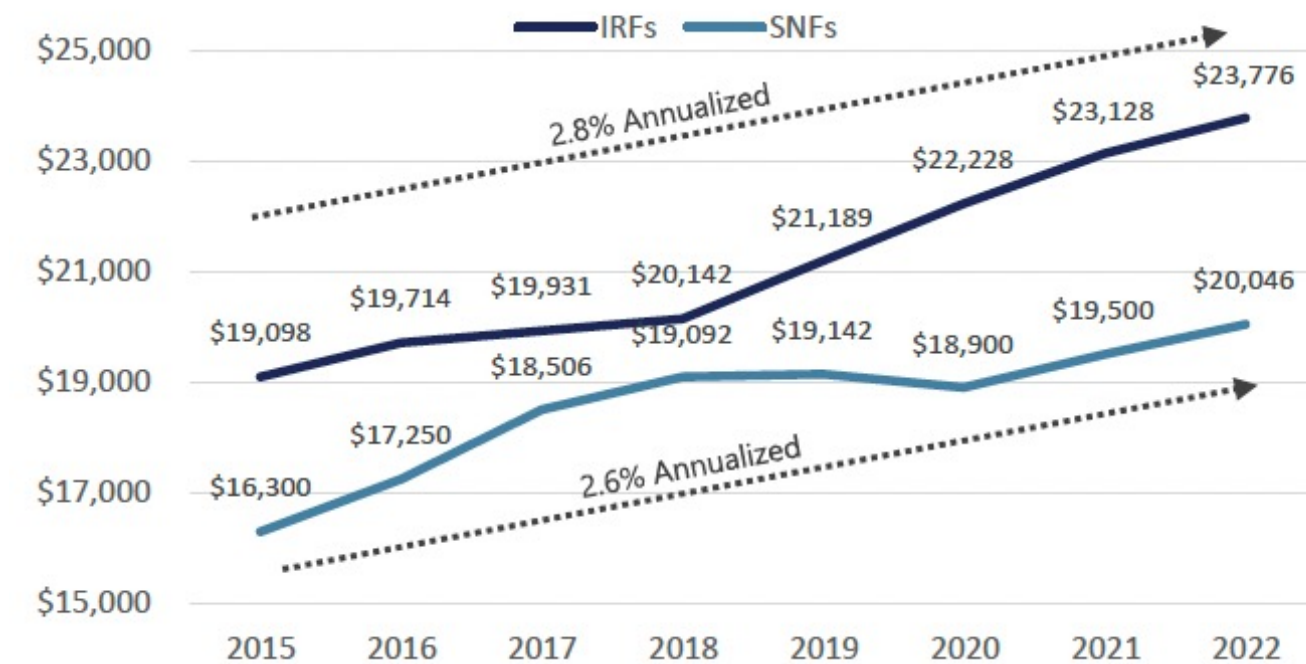
IRF Payor Mix



Patient-Driven Payment Model” Presents Growth Opportunity

- Oct-2019: SNF industry adopted new Medicare reimbursement system called the Patient-Driven Payment Model (PDPM).
- PDPM pays lower rates for patients needing therapy and higher rates for patients needing complex nursing care.
- In response to this financial incentive, SNFs laid off therapists and began actively recruiting patients with complex medical needs.
- Prior to PDPM, SNFs provided higher therapy intensity in order to receive higher reimbursement, but now there is no longer incentive.
- IRF industry benefits from patients who would have gone to a SNF prior to 2019 are now IRF targets.

IRF vs SNF Freestanding Medicare Payment Per Case



IRF Market Overview



PREVENTIVE

Routine health care (screenings, check-ups, patient counseling) to prevent illnesses, disease, or other health problems





AMBULATORY

Medical care delivered on an outpatient basis (doctor visits, walk-in clinics, blood tests, xrays, endoscopy, certain biopsies, certain surgical procedures)





ACUTE

Medical treatment of diseases or injuries for which a patient receives inpatient treatment for a brief but severe episode of illness





POST-ACUTE

Medical care provided after a period of acute care, including: long-term acute care (LTACH), inpatient rehabilitation (IRF), skilled nursing (SNF), home health (HH)

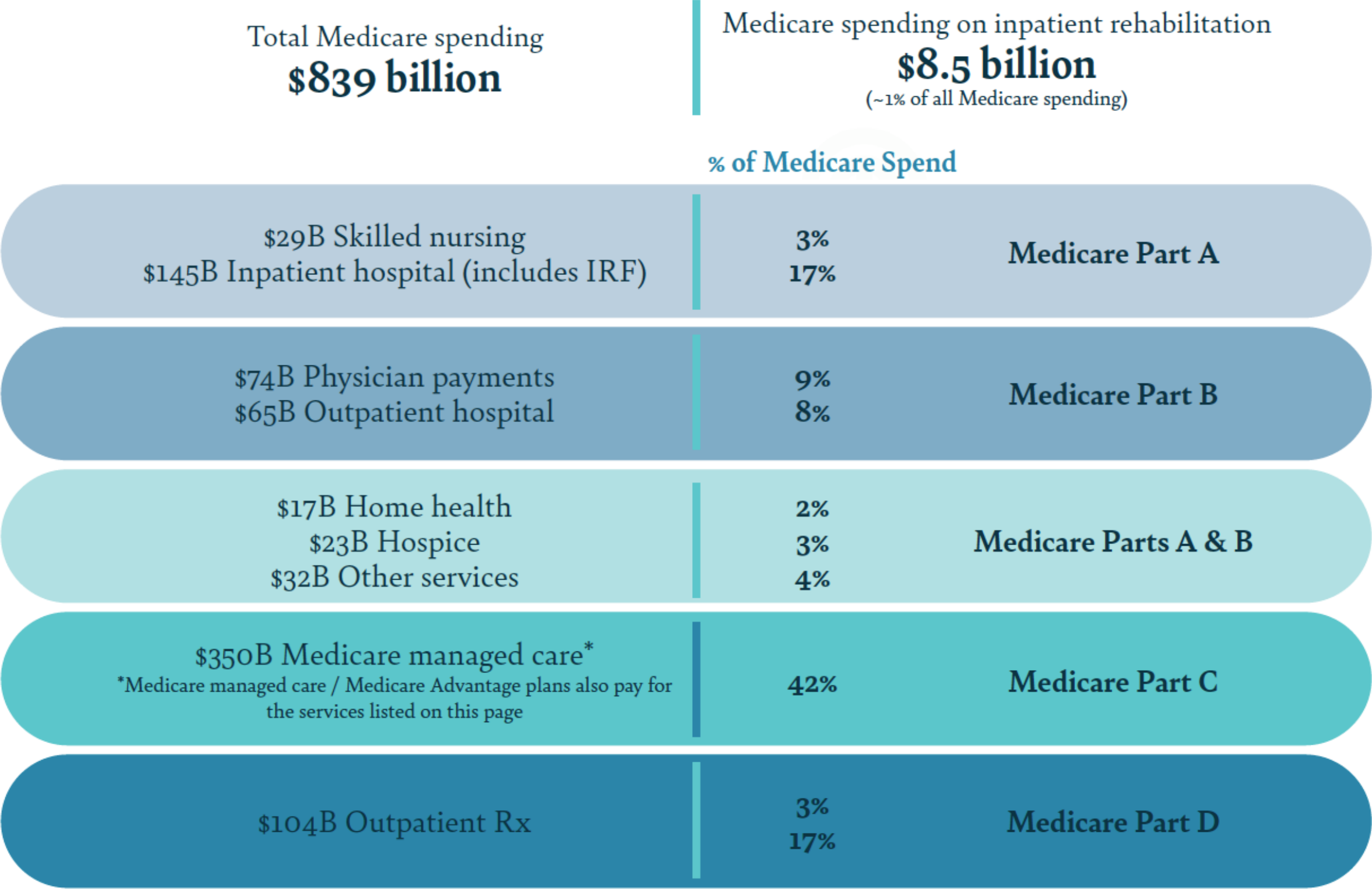
The continued graying of America with some 77 million aging Baby Boomers, a national healthcare system that accounted for \$4.3 trillion of spending in 2021, a finite and virtually unchanged supply of Inpatient Rehabilitation Facilities (IRF) since 2010, and continued disintermediation of Skilled Nursing Facilities (SNF) all point to magnetic growth in the “Post-Acute” healthcare sector.



Medicare acute care patients discharge destination*

LTACH -1%	SNF -19%
IRF -4%	HH -22%
Hospice -4%	
No post-acute care -45%	

MEDICARE SPENDING



- For 2021, total Medicare spending amounted to \$839 billion
- Inpatient rehabilitation spending accounted for \$8.5 billion or roughly 1% of all Medicare spending.
- Inpatient hospital (includes IRF) spending accounted for \$145 billion or 17% of total Medicare Part A spending.

In 2021, inpatient rehabilitation spending accounted for \$8.5 billion or roughly 1% of all Medicare spending

Source: Centers for Medicare & Medicaid Services, Medicare Trustees' Report 2022 – page 12; and MedPAC, Medicare Payment Policy, March 2023 – pages 203, 237, 259 and 285.

Location Overview

Property Information

Address	6301 District Avenue, Little Rock, AR, 72205
Land Area	2.9 Acres 125,453 square feet
Building Size	61,819 square feet
Estimated Completion	TBD 2027
Beds	48
Occupancy	100%
NOI (F-12)	\$3,792,386
Rent Escalation	1.50% Annually
Lease type	Absolute NNN
Ownership	Fee Simple

Overview

- Build-to-suit 61,819 square foot, 48-bed inpatient rehabilitation facility location in Little Rock, AR.
- Purpose-built facility expected to start construction in **Q4 2025** and complete in **Q3 2027**
- The facility is 100% pre-leased to Little Rock Rehabilitation Hospital, LLC on a 20-year absolute NNN lease.
- State-of-the-art facility features all private patient rooms, large windows, advanced rehabilitation gym, on-site café and outdoor courtyard.
- Strategically located along I-630, a major route connecting Downtown Little Rock and Midtown with an average daily traffic that exceeds 112,000 vehicles.
- Located ~1.5 miles from the largest concentration of hospitals in Arkansas.



Market Overview | Little Rock

STRATEGIC LOCATION IN ARKANSAS' HEALTHCARE EPICENTER

The proposed site at I-630 and N. University Avenue is at the epicenter of Arkansas' medical infrastructure, within Little Rock's Midtown Medical Corridor. This intersection offers unmatched connectivity, high visibility, and immediate adjacency to the state's largest concentration of acute care hospitals and medical employers.



HOSPITAL DENSITY & REFERRAL BASE

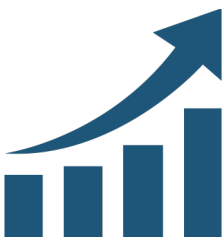
The Midtown Corridor features **over 2,800 licensed hospital beds within a 2-mile radius**, creating one of the **most concentrated acute care clusters in the state**. Major institutions include: Baptist Health Medical Center (834 beds), UAMS Medical Center (514 beds), CHI St. Vincent Infirmiry (615 beds), Arkansas Children's Hospital (336 beds), McClellan VA Hospital (280 beds). Combined, these facilities generate **tens of thousands of discharges annually**, a high percentage of which are eligible for IRF-level post-acute care (e.g., stroke, neurological, orthopedic, trauma). Across the broader **Little Rock MSA**, there are an **estimated 4,000–4,300 total licensed beds**, further solidifying Midtown as the most robust medical referral zone in Arkansas.



UNDERSERVED IRF MARKET

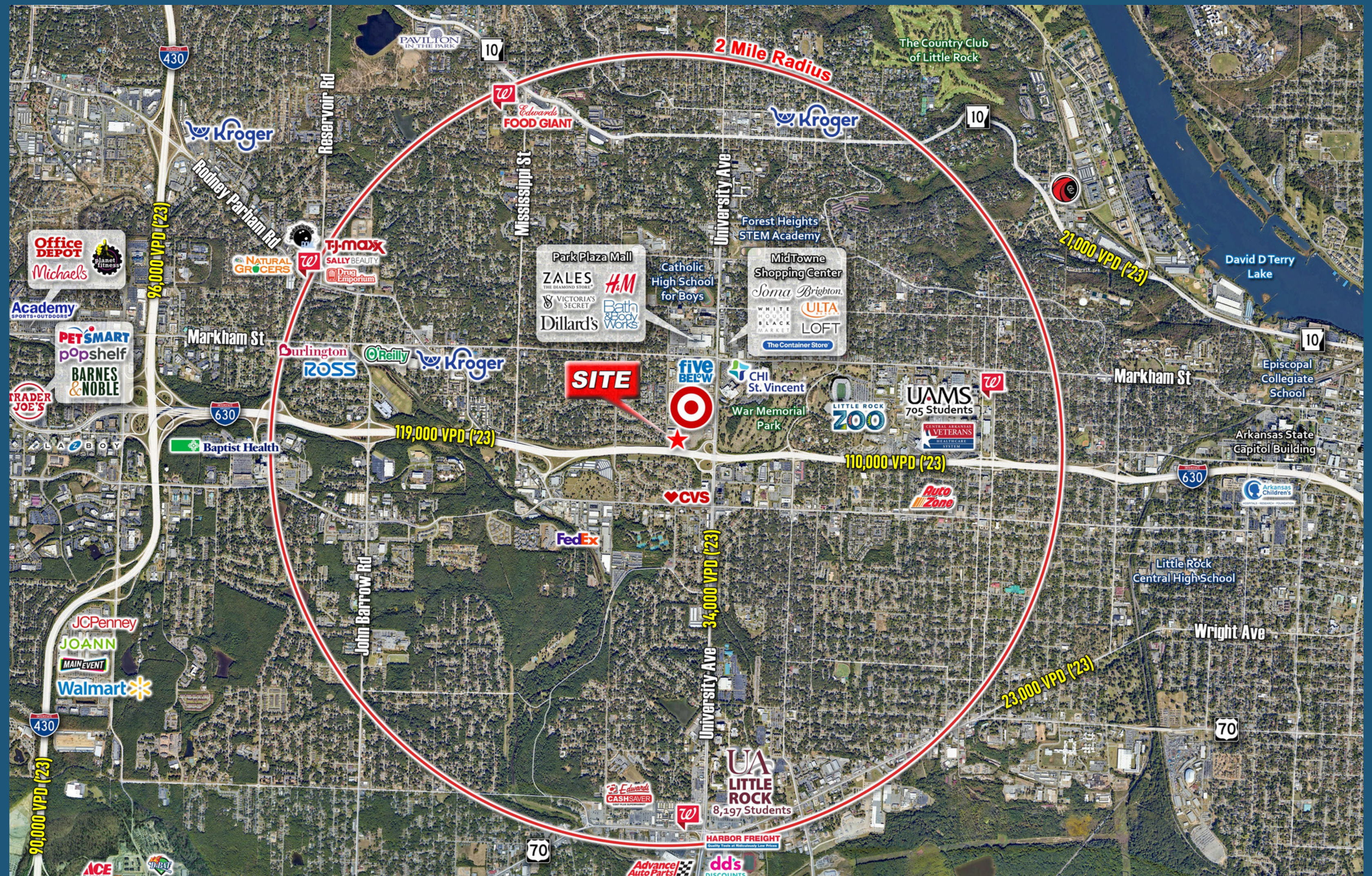
Despite this hospital density, Midtown **lacks any freestanding IRF**. Most rehab care is delivered in hospital-based units, such as Baptist Health's IRF program, which are capacity-limited and lack the patient experience and efficiency of dedicated, purpose-built IRFs.

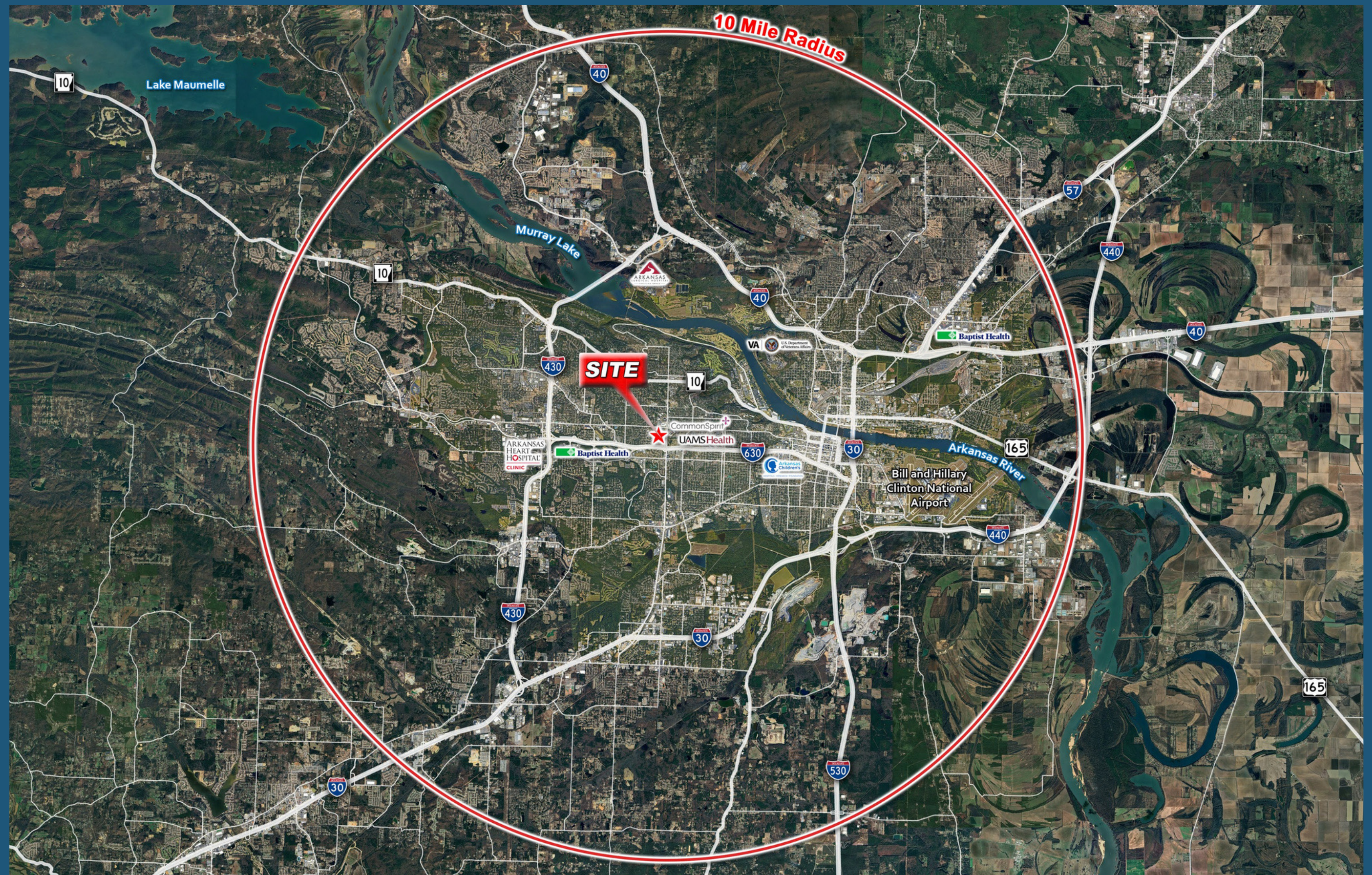
Little Rock has **no modern, freestanding IRF facilities** in its urban core, despite having the demographic and medical profile to support **multiple high-acuity rehab centers**.



STRONG REGIONAL DEMAND DRIVERS

- **Aging Population:** Arkansas' 65+ population is expanding rapidly, driving IRF utilization upward.
- **Limited IRF supply statewide:** Just a handful of IRFs exist in Arkansas, with most located outside the urban core or embedded within hospitals.
- **Disintermediation of SNFs:** National trends show migration from skilled nursing to IRFs due to shorter length of stay and better clinical outcomes (68% discharge-to-community rate for IRFs vs. 44% for SNFs).







**THE DISTRICT
AT MIDTOWN**



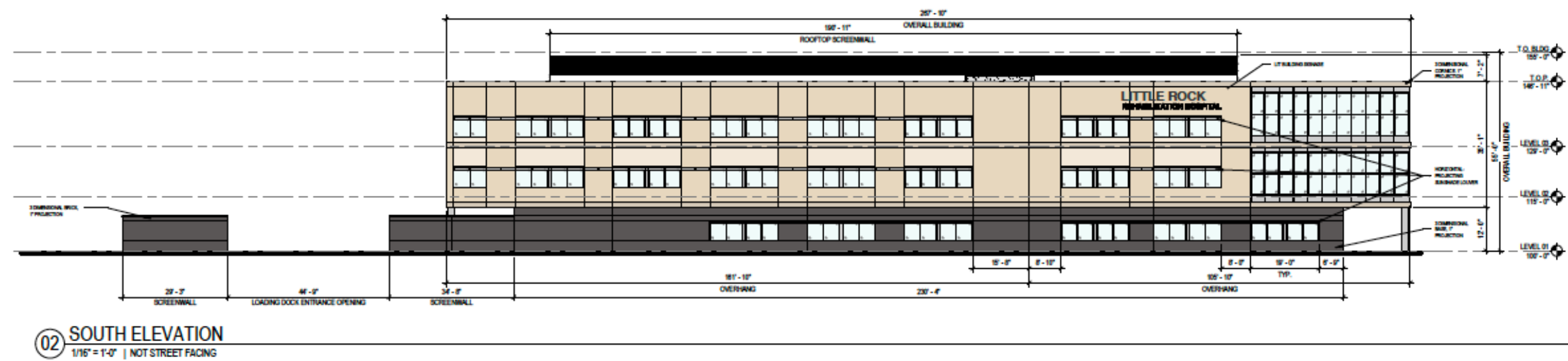


① ARCHITECTURAL SITE PLAN
1" = 20'-0"

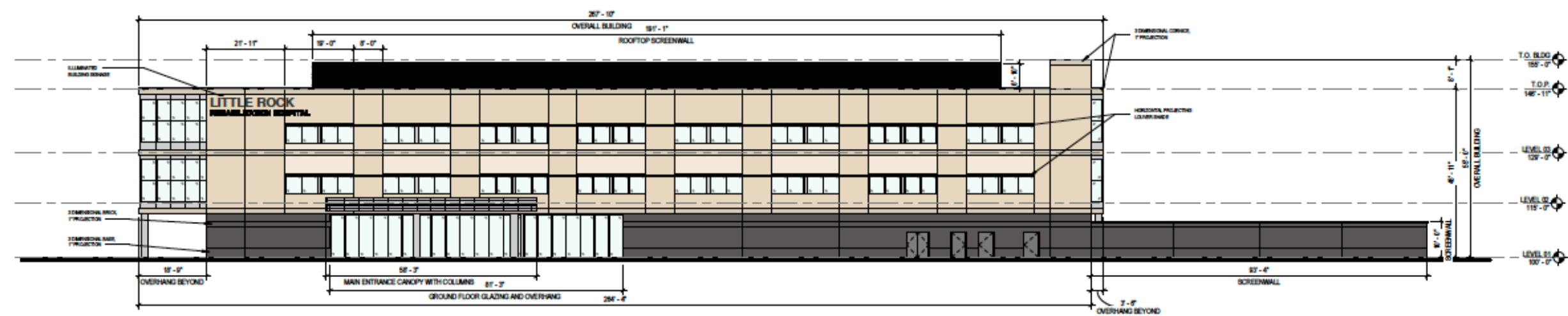
LITTLE ROCK FLOOR PLAN

Perkins&Will

EXTERIOR FINISH LEGEND			
			
			
			



02 SOUTH ELEVATION
1/16" = 1'-0" | NOT STREET FACING



01 NORTH ELEVATION
1/16" = 1'-0" | STREET FACING



Level 1

- | | |
|--|--|
| EXAMPATIENT ROOM | GYM |
| VERTICAL CIRCULATION | SUPPORTING AREA |
| DAYROOM | PUBLIC AREA |
| ADMIN | DINING AND KITCHEN |



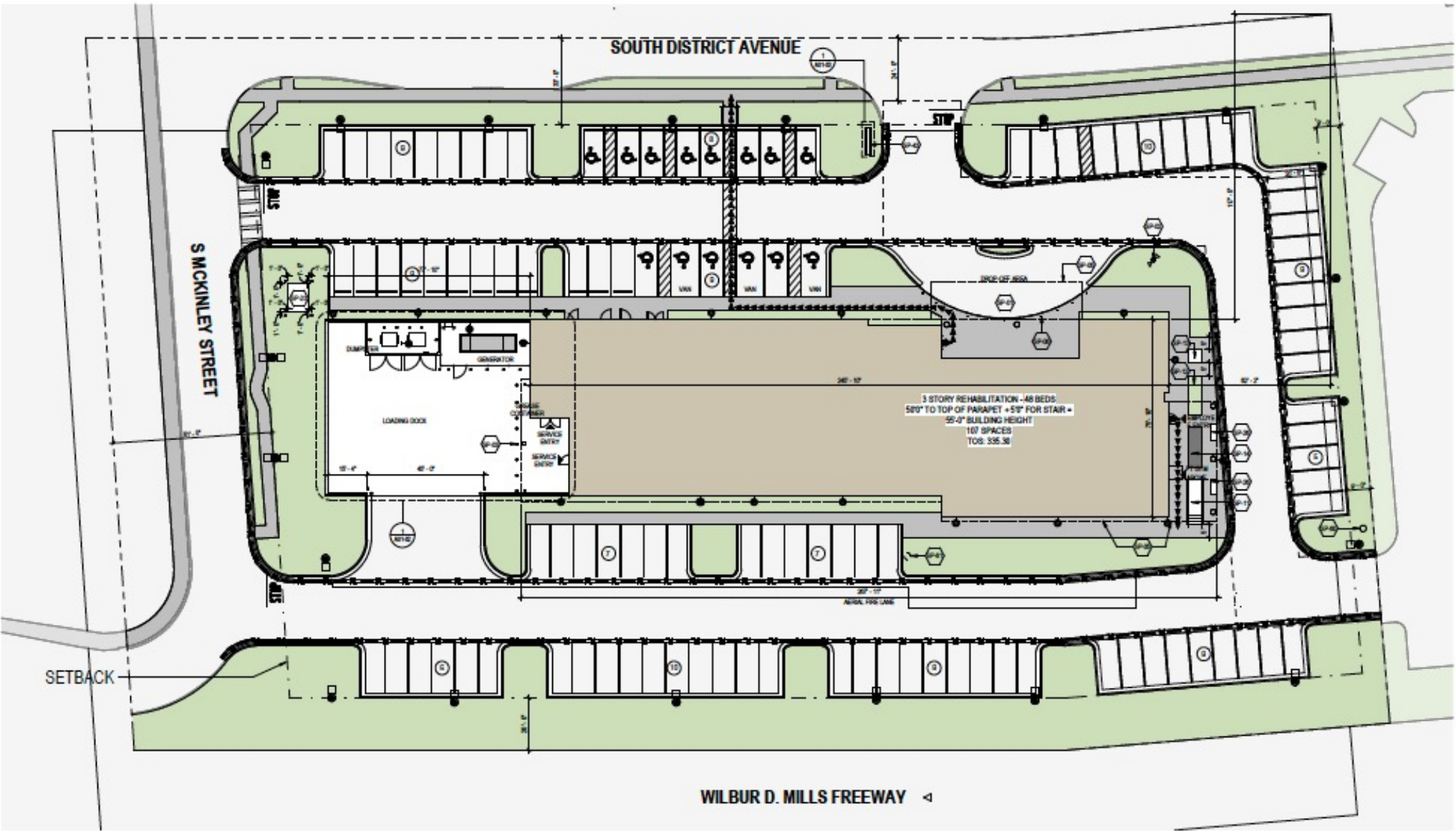
Level 2



Level 3

SITE DATA		PARKING SPACES REQUIRED	PARKING SPACES PROVIDED
PURPOSE LOT AREA ZONING	HOSPITAL 300 AC PDC (PLANNED COMMERCIAL DISTRICT)	PARKING REQUIRED (PARKING DATA) REQUIRED: 110 SPACES (PDC, 20%) ACCESSIBLE PARKING REQUIRED: 20% OF PATIENT AND VISITOR SPACES (NEAREST OR VISITOR PARKING 3.2%) 1 SPACE FOR 1 TO 25 SPACES (25 EMPLOYEE SPACES) TOTAL NEW SPACES REQUIRED 12 ACCESSIBLE SPACES	SPACES @ BUILDING STANDARD SPACES ACCESSIBLE SPACES TOTAL NEW SPACES 110 SPACES 12 SPACES PROVIDED 98 SPACES PROVIDED
BUILDING SETBACK FRONT REAR SIDE		NA NA NA	NA NA NA

SITE PLAN GENERAL NOTES	
1. ARCHITECTURAL SITE PLAN IS ISSUED FOR REFERENCES ONLY. DO NOT SCALE THE DRAWING. IN CASE OF DISCREPANCY, OR IF DIMENSIONS ARE IN QUESTION, OBTAIN CLARIFICATION FROM ARCHITECT PRIOR TO PROCEEDING WITH THE WORK.	
2. RE: CIVIL DRAWINGS FOR ALL SITE CONSTRUCTION INFORMATION INCLUDING BUT NOT LIMITED TO THE FOLLOWING: - SITE LAYOUT AND DIMENSIONAL CONTROL - SITE UTILITIES LOCATIONS OF CONNECTIONS OF SITE UTILITIES TO BUILDING UTILITIES - WATER METER, LOCATIONS AND DETAILS - FAS (FIRE ALARM) CONNECTION LOCATIONS AND DETAILS - ELECTRICAL MANHOLE AND PULL BOX - LOCATIONS AND DETAILS - LOCATION OF 16 IN. OF PERIMETER FOUNDATION DRAINAGE SYSTEM TO SITE STORMWATER SYSTEM - SITE GRADING AND DRAINAGE - STORM DRAINAGE MAINS AND STRUCTURES - LOCATIONS AND DETAILS - DISTURBANCE WARNING SURFACES - LOCATIONS AND DETAILS - TRAFFIC AND PARKING SIGNAGE - LOCATIONS AND DETAILS - PARKING STRIPING AND CURB MARKINGS - TYPICAL ACCESSIBLE SIGNAGE & PARKING SPACES - ACCESSIBLE PARKING SPACE PARKING MARKINGS - TYPICAL HANDICAP SIGNAGE SPACES - PARKING WHEEL STOPS	
3. RE: ELECTRICAL DRAWINGS FOR SITE INFORMATION INCLUDING, BUT NOT LIMITED TO THE FOLLOWING: - SITE LIGHT POLE LOCATIONS, CIRCUIT, AND FIXTURE TYPE DESIGNATIONS - INCANDESCENT SIGN LIGHTING LOCATIONS, CIRCUIT, AND FIXTURE TYPE DESIGNATIONS - FLUO. POLE LIGHTING LOCATION, CIRCUIT, AND FIXTURE TYPE DESIGNATION - STRUCTURE BUILDING ILLUMINATION - ELECTRICAL SERVICES - LOCATIONS AND SIZES - TRANSFORMER LOCATION - TRANSFORMER INFO - ELECTRICAL MANHOLE AND PULL BOX LOCATIONS - PULL BOX LOCATIONS FOR VARIOUS WIRE ELECTRICAL ELEMENTS POWER TO IRRIGATION CONTROLLER	
4. REFERENCE IRRIGATION IN LANDSCAPE DRAWINGS FOR: - IRRIGATION PLANTING AND DETAILS - SEEDING AND SOILING LOCATIONS - IRRIGATION WELLS/SPRINKLER - HANDICAP LAYOUT AND DETAILS (SPECIALTY FIRMING)	
5. REFERENCE IRRIGATION IN LANDSCAPE DRAWINGS FOR: - HEAD AND VALVE LOCATIONS AND SIZES - IRRIGATION PIPING LOCATIONS AND SIZES - IRRIGATION SYSTEM CONTROLLER LOCATION - IRRIGATION DETAILS & SPECIFICATIONS - IRRIGATION CONTROLLER LOCATION - POWER TO IRRIGATION CONTROLLER	
6. *TOP OF SLAB ELEVATION AS REFERENCED THROUGHOUT THE CONTRACT DOCUMENTS DOES NOT NECESSARILY EQUATE TO FINISH FLOOR ELEVATION.* *TOP OF SLAB AS REFERENCED HEREINAFTER IS DEFINED AS THE FINAL POURED CONCRETE SUBFLOOR ELEVATION PRIOR TO APPLICATION OF SCHEDULED FLOOR FINISHES. *FINISH FLOOR ELEVATION IS MEASURED TO TOP OF SCHEDULED FLOOR FINISH MATERIAL.	
ARCHITECTURAL SITE PLAN LEGEND	
NEW PARKING NEW HOSPITAL FOOTPRINT NEWLY SOCCER OR LANDSCAPED AREA, RE: CIVIL AND LANDSCAPE PROPERTY LINE EASEMENT & ROW LANDSCAPE STRACK CONCRETE JOINT 7" CURB NEW PARKING SPACE COUNT HANDICAP SIGN FIRE LANE PATH REQUIRED ACCESSIBLE PATH PARKING LOT LIGHT RE: ELECTRICAL WALL MOUNTED LIGHT FIXTURE RE: ELECTRICAL SLOPED PARKING, RE: CIVIL STREET CROSSING PATH	
SITE PLAN NOTES BY NUMBER	
1. *** Indicates Street Keynote on Plan	
2. PRELIMINARY LANDING ZONE	
3. METAL BOLLARDS, PAINTED, RE: CIVIL	
4. LINE OF BUILDING ACTION	
5. LOT BOLLARD, RE: ELECTRICAL	
6. CAMP, RE: LANDSCAPE	
7. TRAIL, RE: LANDSCAPE	
8. IF STAFF, RE: LANDSCAPE	
9. DISCOURAGED WALKWAY, RE: LANDSCAPE	
10. LOCATION OF TRANSFORMER	
11. SIGN WITH NAME, RE: LANDSCAPE	
12. PREPARED MONUMENT SIGN, RE: BEST ADVISE FOR MORE INFORMATION	
13. NEW LOCATION FOR EXISTING POLE SIGN, BEARING IN MIND TO PROVIDE FOUNDATION FOR POLE SIGN, ELECTRICAL REQUIREMENTS TO BE RELOCATED BY BEARING WAREHOUSE	
14. EXISTING LANDSCAPING CONTRA POLE SIGN TO BE RELOCATING	



1 ARCHITECTURAL SITE PLAN
1" = 20'-0"

Perkins&Will

2201 River St., Suite 200
Dallas, TX 75201
214.261.8000
perkinswill.com

CONSULTANTS

MEP
DBI ENGINEERING CONSULTANTS
5000 QUAKER DRIVE, SUITE 400
DALLAS, TX 75244
STRUCTURAL
STANTEC
8001 TERRY PARKWAY, SUITE 200
PLANO, TX 75074
CIVIL
FORESTE GROUP, LLC
2011 MARCOLA AVENUE, SUITE
100 BIRMINGHAM, AL 35205
LANDSCAPING
FORESTE GROUP, LLC
2011 MARCOLA AVENUE, SUITE 100
BIRMINGHAM, AL 35205
ROAD SERVICE
SCS&S DESIGN SOLUTIONS
2001 LONG PRAIRIE RD, SUITE 107 727
FLOWER MOUND, TX 75022

CONTRACTOR

OPERATOR
HOBBS REHABILITATION PARTNERS, LLC
400 CENTURY PARKWAY, SUITE 200
ALLIANCE, TEXAS 76010
OWNER
KENNOR CROSS INVESTMENTS, LLC
400 MARSH RIDGE ROAD
CARROLLTON, TX 75006
FACILITY
LITTLE ROCK REHABILITATION HOSPITAL
6301 DISTRICT AVENUE
LITTLE ROCK, AR 72205



FACILITY:
LITTLE ROCK
REHABILITATION
HOSPITAL
6301 DISTRICT AVENUE
LITTLE ROCK, AR 72205

DEVELOPER:

KENNOR CROSS
INVESTMENTS, LLC
4332 MARSH RIDGE ROAD
CARROLLTON, TX 75010

KEY PLAN



ISSUE CHART

NO.	DATE	DESCRIPTION
1	03.06.2025	ISSUED FOR CONSTRUCTION

ARCHITECTURAL SITE
PLAN

SHEET NUMBER

A01-01

Opportunity Financials



Project Budget | Sources & Uses

Uses:

Site Acquisition:

Land Acquisition	\$	4,077,216
Total Site Acquisitions	\$	4,077,216

Hard Costs

Building Hard Costs 61,819 SF	\$	24,334,852
Builders Risk/GL Insurance	\$	65,000
Construction Mgt Fee	\$	365,023
Contingency	\$	1,238,244
Total Hard Costs	\$ 420.63	\$ 26,003,119

Soft Costs

Total design / studies / surveys / pre-dev	\$	1,419,155
Total enviro / soil / testing	\$	22,150
Total ahj / expediting / use fees	\$	115,267
Total utility / taps / etc.	\$	66,774
Development Fee	\$	1,321,906
Appraisals	\$	20,000
Legal	\$	50,000
Financing Related Fees	\$	758,477
Construction Interest / Operating Reserves	\$	3,316,798
Closing Costs & Title Policy	\$	189,619
Property Taxes	\$	85,622
Transfer/Other Taxes	\$	13,455
Bank Inspections & Insurance	\$	120,000
Travel	\$	14,000
Misc/Other	\$	19,750
Soft Cost Contingency	\$	310,552
Total Soft Costs	\$ 126.88	\$ 7,843,525
Total Project Costs	\$ 613.47	\$ 37,923,860

Sources & Uses

Sources:

Bank Construction Loan	\$	30,339,088
Preferred Equity	\$	5,688,579
Sponsor Equity	\$	1,896,193
Total Sources	\$	37,923,860

Uses:

Site Acquisition:	\$	4,077,216
Total Hard Costs:	\$	26,003,119
Total Soft Costs:	\$	7,843,525
Total Uses	\$	37,923,860

Rent Schedule

Year	Escalations	Annual Rent	Rent PSF
Year 1		\$3,792,386	\$61.35
Year 2	1.5%	\$3,849,272	\$62.27
Year 3	1.5%	\$3,907,011	\$63.20
Year 4	1.5%	\$3,965,616	\$64.15
Year 5	1.5%	\$4,025,100	\$65.11
Year 6	1.5%	\$4,085,477	\$66.09
Year 7	1.5%	\$4,146,759	\$67.08
Year 8	1.5%	\$4,208,960	\$68.09
Year 9	1.5%	\$4,272,095	\$69.11
Year 10	1.5%	\$4,336,176	\$70.14
Year 11	1.5%	\$4,401,219	\$71.20
Year 12	1.5%	\$4,467,237	\$72.26
Year 13	1.5%	\$4,534,246	\$73.35
Year 14	1.5%	\$4,602,259	\$74.45
Year 15	1.5%	\$4,671,293	\$75.56
Year 16	1.5%	\$4,741,363	\$76.70
Year 17	1.5%	\$4,812,483	\$77.85
Year 18	1.5%	\$4,884,670	\$79.02
Year 19	1.5%	\$4,957,940	\$80.20
Year 20	1.5%	\$5,032,309	\$81.40
Year 21	1.5%	\$5,107,794	\$82.62
Year 22	1.5%	\$5,184,411	\$83.86
Year 23	1.5%	\$5,262,177	\$85.12
Year 24	1.5%	\$5,341,110	\$86.40
Year 25	1.5%	\$5,421,226	\$87.70
Year 26	1.5%	\$5,502,545	\$89.01
Year 27	1.5%	\$5,585,083	\$90.35
Year 28	1.5%	\$5,668,859	\$91.70
Year 29	1.5%	\$5,753,892	\$93.08
Year 30	1.5%	\$5,840,201	\$94.47

Location	6301 District Avenue, Little Rock, AR, 72205
Tenant	Nobis Rehabilitation Partners
Guarantor	Nobis Rehabilitation Partners
Property Use	Inpatient Rehabilitation Hospital
Year Built	2027
Rentable Square Footage	61,819
Occupancy	100%
Initial Lease Term	20 Years
Lease Commencement	TBD (upon completion)
Rent Commencement	3 Months of Abatement
Renewal Options	Two Five-Year Options
Rent Year 1	\$3,792,386
Rent PSF	\$61.35
Rental Increase	1.5% Annual Escalations
Lease Structure	Absolute NNN

Representative Nobis Rehabilitation Hospital - OKC

